

Surgical Pathology Report

FINAL DIAGNOSIS

A) Ureter, left, distal margin, biopsy:

- Negative for malignancy

B) Ureter, right, distal margin, biopsy:

- Negative for malignancy

C) Urethra, margin, biopsy:

- Negative for malignancy

D) Bladder, cystectomy:

- High-grade urothelial carcinoma with focal squamous differentiation (WHO grade 3/3), 2.9 cm greatest dimension, invasive through the muscularis propria and into perivesical adipose tissue, with perineural invasion (see microscopic description)
- Focal high-grade urothelial carcinoma in-situ of bladder dome
- Extensive follicular cystitis
- Granulation tissue and chronic inflammation with urothelial regeneration, consistent with previous biopsy site

E) Lymph node, left obturator, dissection:

- One lymph node negative for malignancy (0/1)

F) Lymph node, left external iliac, dissection:

- One lymph node negative for malignancy (0/1)

G) Lymph node, right external iliac, dissection:

- One lymph node negative for malignancy (0/1)

H) Lymph node, right obturator, dissection:

- Nine lymph nodes negative for malignancy (0/9)

I) Lymph node, right common iliac, dissection:

- Three lymph nodes negative for malignancy (0/3)

J) Lymph node, left common iliac, dissection:

- Four lymph nodes negative for malignancy (0/4)

K) Lymph node, right internal iliac, dissection:

- One lymph node negative for malignancy (0/1)

ICD-O-3
Carcinoma, urothelial NOS
8/20/3
Site Lateral wall of bladder
C67.2
JW 6/17/13
Per TSS, squamous differentiation is ~40%.

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CLINICAL HISTORY

-year-old with hematuria with left bladder wall tumor.

MICROSCOPIC DESCRIPTION

Specimen: Bladder

Procedure: Radical cystectomy

Tumor Site: Left lateral wall

Tumor Size

Greatest dimension: 2.9 cm

Additional dimensions: 2.5 x 1.5 cm

Histologic Type: Urothelial (transitional cell) carcinoma with focal squamous differentiation

Associated Epithelial Lesions: Urothelial carcinoma in-situ

Histologic Grade: High-grade

Tumor Configuration: Solid, Infiltrating

Microscopic Tumor Extension: Perivesical fat

Margins: Margins uninvolved by invasive carcinoma

Distance of invasive carcinoma from closest margin: 5 mm

Specify margin: Radial

Perineural invasion: present

Lymph-Vascular Invasion: Indeterminate

Staging:

Primary Tumor (pT): pT3a: Tumor invades perivesical tissue microscopically

Regional Lymph Nodes (pN): pN0: No lymph node metastasis

Number of Lymph Nodes Examined: 20

Number of Lymph Nodes Involved: 0

Distant Metastasis (pM): pMX (Not applicable)

Additional Pathologic Findings: Inflammation/regenerative changes

GROSS DESCRIPTION

Received are 11 specimens, labeled [REDACTED]

A) Received fresh and designated "left distal ureter," is a 0.9 x 0.2 x 0.2 cm red, soft, tissue with ureter lumen, submitted in its entirety in cassette (FSA1) for intraoperative diagnosis.

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B) Received fresh and designated "right distal ureter," is a 1 x 0.4 x 0.2 cm red, soft tissue with ureter lumen, submitted in its entirety in cassette (FSB1) for intraoperative diagnosis.

C) Received fresh designated "urethral margin," inked distal – non-inked proximal," is a 1.8 x 1 x 0.3 cm soft tissue with urethral lumen. Submitted in its entirety in cassette (FSC1) for intraoperative diagnosis.

D) Received in formalin designated "bladder," is a bladder with 12 x 8 x 1 cm of perivesicular fat. The bladder dome to urethra is 5.5 cm right to left is 10 cm and anterior to posterior is 5 cm. A 2 x 1.4 cm tan ulcerative lesion with evidence of prior biopsy is identified in the anterior lateral left bladder wall, 2.4 cm from the urethral margin and 3 cm from the left ureter orifice. The posterior surgical margin adjacent to the mass is inked black. On the cut surface the mass is firm and tan and extends 1.5 cm from the bladder mucosa into the wall and detrusor muscles. The mass is 0.1 cm from the posterior soft tissue margin. After sectioning, the mass measures 2.5 x 1.5 x 2.9 cm. Representative sections are submitted as follows:

- (D1) Tumor to soft tissue margin
- (D2-D3) Sections of tumor
- (D4) Tumor to closest adjacent mucosa
- (D5) Right ureteral orifice
- (D6) Left ureteral orifice
- (D7) Bladder neck
- (D8) Dome
- (D9) Right wall
- (D10) Left wall
- (D11) Posterior wall
- (D12) Anterior wall
- (D13) Trigone
- (D14-D22) perivesicular fat

E) In formalin, labeled "left obturator node," are 3 cm of lobular fatty tissue with an interspersed lymph node which is fusiform, 1 cm in greatest dimension. The lymph node is submitted in its entirety as (E1).

F) In formalin, labeled "L external iliac lymph node," is a lobular fatty tissue, 5.5 x 3.5 x 1.5 cm, with a central lymph node, 3.9 x 2.5 x 0.9 cm. The lymph node is submitted in its entirety as (F1-F5).

G) In formalin, labeled "R external iliac LN," is a 4 x 2.9 x 1.1 cm lymph node with abundant attached fat. The lymph node is submitted in its entirety as (G1-G5).

H) In formalin, labeled "right obturator lymph node," are 4 x 3 x 1.5 cm with lobular fatty tissues with interspersed lymph nodes. The lymph nodes range from 0.6 cm to 1.5 cm. The lymph nodes are submitted in their entirety as follows:

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- (H1) Two lymph node candidates
- (H2) Two lymph node candidates
- (H3) One bisected lymph node
- (H4) Multiple lymph node candidates

I) In formalin, labeled "R common iliac lymph node," are fatty tissues, 2.5 cm in aggregate, with interspersed lymph nodes up to 1.5 cm. The lymph nodes are submitted in toto, one block as (I1).

J) In formalin, labeled "L common iliac lymph nodes," are multiple lymph nodes up to 1.1 cm with abundant attached fat. The lymph nodes are submitted as (J1-J2) in toto.

K) In formalin, labeled "R internal iliac LN," is a 2.9 x 2.5 x 0.9 cm encapsulated nodule suggestive of a lymph node with moderate attached fat. The cut surface is pink and homogeneous. In its entirety as (K1-K3).

Notes	Yes	No
Diagnosis Discrepancy		
Primary Tumor Site Discrepancy		
ICD-9 Discrepancy		
ICD-10 Discrepancy		
ICD-11 Discrepancy		
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TCGA Pathologic Diagnosis Discrepancy Form 4.05

Study Subject ID:		Person ID: N/A
Study/Site:	TCGA Bladder Urothelial Carcinoma - (Bladder Urothelial Carcinoma)	Age: N/A
Event:	PathDiscrepancy	Date of Birth:
Interviewer:		Sex: F

Tumor Identifier Provided on Initial Case Quality Control Form		Provide the tumor identifier documented on the initial case quality control form for this case.
Pathologic Diagnosis Provided on Initial Pathology Report	Focal squamous differentiation	Provide the diagnosis/ histologic subtype(s) documented on the initial pathology report for this case. If the histology for this case is mixed, provide all listed subtypes.
Histologic features of the sample provided for TCGA, as reflected on the CQCF	40% squamous	Provide the histologic features selected on the TCGA Case Quality Control Form completed for this case.
Discrepancy between Pathology Report and Case Quality Control Form		
Provide the reason for the discrepancy between the pathology report and the TCGA Case Quality Control Form	Section submitted to TCGA had more squamous differentiation than the whole tumor submitted for permanent.	Provide a reason describing why the diagnosis on the initial pathology report for this case is not consistent with the diagnosis selected on the TCGA Case Quality Control Form.
Name of TSS Reviewing Pathologist or Biorepository Director	Provide the name of the pathologist who reviewed this case for TCGA.	